

Section 11

Auditor of State Findings & Financial Oversight

Version 2.0 — Preliminary Draft

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SECTION 11 — AUDITOR FINDINGS & FINANCIAL OVERSIGHT

Petition to Audit: Iowa Department of Health and Human Services

Child Welfare Systems — Longitudinal Accountability Record

Version 2.3 | June 5, 2026

SECTION PURPOSE

This section documents financial oversight deficiencies, procurement monitoring failures, and recurring fiscal accountability concerns identified by the Iowa Auditor of State's own office across a 13-year documented record. The significance of this section is structural: the findings documented here were identified not by external critics or adversarial parties, but by the State of Iowa's own independent audit authority — the office being petitioned to conduct this review.

The existence of repeat findings, acknowledged monitoring gaps, and documented federal grant accountability failures within the Auditor of State's own prior reporting establishes that the fiscal and contractual accountability infrastructure of Iowa DHHS has been independently identified as deficient. This section presents those findings in their sourced form as primary evidentiary support for Audit Objective 3 (financial controls and procurement compliance) and Audit Objective 4 (programmatic accountability).

Cross-reference: Section 10 (external evaluations — Sivic Solutions/IV-E, CWPPG, C!A); Section 12 (Ombudsman complaint patterns); Section 13 (Systemic Failure Matrix).

11.1 ATTACHMENT A22 — CONSOLIDATED AOS REPORTS OF RECOMMENDATIONS

INDEXED SERIES (2009–2021)

Source: Iowa Auditor of State | Annual Reports to Iowa DHS | 2009–2021

Primary Authority: Iowa Code § 11.6 | Audit jurisdiction — DHS

Evidentiary Classification: Primary | Domains: A; B; C; D; E; F; G; H

deficiency categories: F1; F5; F6; F7; F9; F12

11.1.1 Overview and Scope

The Iowa Auditor of State's office issued annual Reports of Recommendations to the Iowa Department of Human Services (now DHHS) covering the period from FY2009 through FY2021. This attachment comprises the indexed series of those reports, representing a 13-year longitudinal record of findings by the state's own independent audit authority.

The 13-year series documents a pattern of persistent, repeat findings across fiscal controls, contract oversight, payment accuracy, and child welfare service delivery monitoring. This pattern is not inferential — it is documented in the Auditor's own published findings, which identify by year and frequency which deficiencies recurred without correction.

11.1.2 Core Finding — 60% Repeat Finding Rate

The most significant structural finding from the indexed series is the documented repeat finding rate. The FY2019 report documented:

"60% of findings were repeat findings from previous years." (AOS Report FY2019)

This 60% repeat finding rate is itself a primary audit-relevant finding. Under GAGAS standards, repeat findings indicate a systemic failure of corrective action — not an isolated or incidental compliance gap. A repeat finding rate of 60% sustained over a 13-year series documents that the agency's internal corrective action mechanisms were not functioning.

11.1.3 Foster Care Visit Compliance — 40.5% Failure Rate

The AOS documented that in FY2011:

"40.5%... did not receive a visit at least once every calendar month." (AOS FY2011)

Federal CFSR standards require regular caseworker contact as a fundamental child safety practice. Iowa's own audit authority documented that over 40% of children in foster care were not receiving federally required monthly visits during the same period the state was under its Round 1 PIP corrective obligations. Cross-reference: Section 6 (CFSR Round 1 PIP — worker visits with child: baseline 10%, goal 25%; identical deficiency documented in federal review).

11.1.4 Concurrent FIP and Foster Care Payments — 19% Rate

"19% received both FIP checks and foster care payments." (AOS FY2013, p.4)

The FY2013 report documents that approximately one in five cases reviewed received both Family Investment Program (FIP) and foster care payments simultaneously — an overlapping payment condition that indicates a failure of basic financial control reconciliation between benefit programs administered by the same agency.

11.1.5 Understated Financial Commitments — \$36.5M Material GAAP Violation

"\$36,524,669 understated commitments." (AOS FY2014)

The FY2014 report identified a material financial reporting failure: \$36.5 million in financial commitments were understated in the agency's reported figures, constituting a GAAP violation. Under Iowa Code § 11.6, the Auditor of State is authorized to audit state agencies' financial records. A material misstatement of this magnitude — in an agency administering federal Title IV-E and IV-B child welfare funds — represents a financial control failure at the institutional level.

11.1.6 Post-Death Medicaid Payments — Documented Across Three Fiscal Years

The AOS series documents payments issued to deceased recipients in three separate fiscal years:

- FY2009: \$23,271 in payments across 255 transactions
- FY2017: \$3.2M in uncollected nursing facility penalties (related)
- FY2020: \$20,061.29 in payments in a single documented case; post-death payments also referenced in Medicaid MCO context

(AOS Annual Reports, respective fiscal years)

Post-death payments represent a baseline failure of identity and eligibility verification — the most fundamental level of financial control. Their recurrence across three fiscal years, separated by a decade, documents that the corrective action implemented following the FY2009 finding did not prevent recurrence in FY2017 or FY2020.

11.1.7 Workforce Attrition Concurrent With Financial Findings

The AOS series documents the workforce context concurrent with these financial findings:

"16% staff loss + 29% caseload increase documented in same period." (AOS series)

This workforce data from the Auditor's own reports is consistent with the CFSR Round 3 PIP workforce documentation (Section 6.4.17.1: 13% decline, 19% caseload increase as of 2019). The AOS series thus independently corroborates the workforce deterioration timeline documented in federal CFSR records, establishing that fiscal oversight capacity degraded alongside service delivery capacity during the same period.

11.1.8 Reporting Lag — FY2020–2021 Reports Released 2024

The AOS FY2020 and FY2021 reports were not publicly released until 2024 — a 3-year reporting lag. The FY2020 report's concluding statement:

"Oversight is watchful and responsible care — DHS failed to provide." (AOS FY2020, p.1)

This statement — made by the Iowa Auditor of State, not by the petitioner — constitutes an independent institutional determination that DHS's oversight capacity had failed. The 3-year delay between the fiscal year covered and public release of that determination represents a secondary accountability gap: the independent audit finding was not available to the public, the legislature, or service recipients for three years.

AUDIT RELEVANCE: The 13-year indexed series establishes a documented pattern of: (1) repeat findings at a 60% rate; (2) material financial misstatements; (3) concurrent benefit payment errors across multiple programs; (4) workforce deterioration concurrent with fiscal compliance failures; and (5) a 3-year public reporting lag. Under GAGAS, these conditions collectively support a finding of systemic deficiency in financial controls, not isolated annual compliance gaps.

11.2 ATTACHMENT A23 — AOS SERVICE CONTRACT MONITORING REPORT

(JULY 1, 2010 – JUNE 30, 2018)

Source: Iowa Auditor of State | Performance Audit Division

Report Date: January 22, 2020

Authority: Iowa Code § 11.6

Evidentiary Classification: Primary | Domains: B; F; H

deficiency categories: F5; F6; F7

11.2.1 Overview and Scope

In January 2020, the Iowa Auditor of State's Performance Audit Division released a special performance audit of DHS service contract monitoring practices covering the 8-year period from July 1, 2010 through June 30, 2018. Forty service contracts were selected for testing.

This report is distinct from the annual Reports of Recommendations series (A22) in that it constitutes a focused, standalone performance audit of the specific mechanism by which Iowa DHS was responsible for ensuring that contracted service providers were meeting their performance obligations. It is not an annual compliance review — it is an 8-year retrospective audit of the monitoring system itself.

11.2.2 Zero Budget Monitoring — All 40 Contracts

"40 contracts tested — zero documentation of budget monitoring." (AOS Report, January 2020)

Across all 40 contracts selected for review spanning an 8-year period, the Auditor found zero documentation that DHS had monitored contractor budgets. Not one contract in the 8-year sample contained evidence of budget monitoring by the administering agency. This is not a marginal finding. Budget monitoring of contracted service providers is a basic internal control requirement under both state procurement law and federal grant conditions applicable to Title IV-B and IV-E funded services.

11.2.3 Zero Communication of Deficiencies to Contractors

"Zero evidence deficiencies communicated to contractors." (AOS Report, p.15)

Across the same 40-contract, 8-year sample, the Auditor found zero documentation that DHS had communicated identified performance deficiencies back to contractors. The monitoring mechanism — to the extent any oversight activity occurred — did not produce feedback to the contracted service providers who were responsible for correcting deficiencies.

AUDIT RELEVANCE: This finding is structurally significant beyond its face value. The C!A Report (2023) independently documented that "DHS cannot track what contracted providers actually deliver" (Section 10, A18). The CWPPG Report (2019) documented that providers reported DHS decisions were made outside Family Team Meetings before court (Section 10, A19). The AOS Service Contract Monitoring Report establishes the mechanism for why: DHS's contract monitoring infrastructure produced no documentation that deficiencies were ever communicated to providers. Independent evaluators in 2019 and 2023 documented the operational consequence of a monitoring system the Auditor confirmed had no corrective communication function.

11.2.4 Zero Corrective Action Plans Required

"Zero corrective action plans required." (AOS Report)

No corrective action plans were required of any contractor across the 40-contract sample. Combined with the finding of zero deficiency communication, this establishes a three-layer monitoring failure: no budget oversight, no deficiency communication, and no corrective action structure for any of the 40 contracts reviewed.

11.2.5 Five Expired Contracts — No Closeout Documentation

"All 5 expired contracts with no closeout documentation." (AOS Report, p.15)

Of the contracts that expired during the review period, none had closeout documentation.

Contract closeout is required under both state procurement procedures and federal grant conditions to document final performance, reconcile funding, and confirm that contracted deliverables were met. The absence of any closeout documentation for expired contracts means there is no auditable record of whether the state received the services for which it paid.

11.2.6 No Performance Evaluations Completed — Ever

"No performance evaluations ever completed." (AOS Report, p.17)

This is the most sweeping finding in the report. Across an 8-year period and 40 contracts, no performance evaluations were completed. Iowa DHS paid contracted service providers — many of whom were delivering child welfare services to vulnerable families — without ever formally evaluating whether those providers were performing.

AUDIT RELEVANCE: The Sivic Solutions Report (2023) (Section 10, A21/A22) identified that \$40M in SFY2023 IV-E prevention claims may require revision because Solution Based Casework (SBC) — a primary contracted service — was found not to meet IV-E eligibility criteria. The AOS Service Contract Monitoring Report documents the structural reason why: Iowa DHS had an 8-year practice of issuing no performance evaluations for contracted services. A monitoring system with no performance evaluations, no deficiency communication, and no corrective action plans cannot detect — and did not detect — that a core contracted service was potentially misclassified for federal reimbursement for over a decade.

11.2.7 Cross-Reference — Sivic Solutions Concurrent Finding

The AOS Service Contract Monitoring Report (covering 2010–2018) and the Sivic Solutions IV-E Recommendations Report (covering 2019–2023) together document a continuous 14-year arc of contract accountability failure:

2010–2018 (AOS): No budget monitoring, no deficiency communication, no performance evaluations, no closeout documentation — for any of 40 tested contracts.

2019–2023 (Sivic): \$40M in IV-E prevention claims requiring revision; zero contractors meeting the Well-Supported program mix requirement; Kinship Navigator not submitted for federal Clearinghouse evaluation; SafeCare at risk of losing IV-E eligibility.

These two independent reports, by different entities at different times, document the same structural deficiency in Iowa's contract accountability infrastructure across a combined 14-year window.

11.3 ATTACHMENT A24 — AOS MEDICAID MCO CONTRACTS REPORT (2016–2019)

Source: Iowa Auditor of State | Medicaid MCO Contract Review

Period: 2016–2019

Authority: Iowa Code § 11.6

Evidentiary Classification: Primary | Domains: B; F; G; H

deficiency categories: F5; F6; F7; F9

11.3.1 Overview and Scope

The AOS conducted a review of Iowa Medicaid Managed Care Organization (MCO) contract administration covering the period 2016–2019. This review examined the state's oversight of managed care contracts covering Medicaid-enrolled children and families, including those involved in the child welfare system.

Iowa's child welfare-involved population is substantially Medicaid-enrolled. Children in foster care are categorically eligible for Medicaid, and families under DHS supervision commonly access mental health, substance abuse, and medical services through the Medicaid program. The oversight quality of MCO contracts is therefore directly relevant to whether the services that DHS safety plans, case plans, and court orders reference as available are in fact being delivered, monitored, and reimbursed accurately.

11.3.2 Payment Accuracy — Post-Death Payments in MCO Context

The Medicaid MCO review identified post-death payments within the managed care framework, consistent with the pattern documented in the annual AOS series (Section 11.1.6). The recurrence of post-death payment findings across both fee-for-service Medicaid (AOS annual series) and managed care (MCO review) contexts establishes that the payment accuracy failure was not isolated to one payment modality — it was present in both.

11.3.3 Contract Monitoring — Parallel Pattern

The MCO contract review identified monitoring deficiencies consistent with those found in the service contract monitoring report (A23). The concurrent monitoring failures across both the contracted service system (A23: child welfare services) and the MCO system (A24: Medicaid managed care) document that Iowa's contract accountability infrastructure was uniformly deficient — not selectively deficient in one program area.

11.3.4 Audit Relevance — Medicaid as Service Delivery Infrastructure

AUDIT RELEVANCE: CFSR Round 3 case review data (Section 6) documented that Item 17 (Physical Health) was rated as a Strength in only 59.09% of applicable cases, and Item 18

(Mental/Behavioral Health) in only 55.81%. These items directly reflect whether children and families are receiving the physical and mental health services that case plans reference. The AOS MCO review documents that the Medicaid managed care contracts through which those services are accessed and paid were themselves subject to monitoring deficiencies. The federal performance failure (CFSR Items 17–18) and the fiscal accountability failure (AOS MCO monitoring) are structurally linked: inadequate contract oversight of the payment mechanism for health services is a plausible contributing cause to inadequate service delivery outcomes at the case level.

11.4 ATTACHMENT A25 — AOS SPECIAL INVESTIGATION — CROSSROADS

MENTAL HEALTH AND SUBSTANCE ABUSE SERVICES (2025)

Source: Iowa Auditor of State | Special Investigation

Year: 2025

Authority: Iowa Code § 11.6

Evidentiary Classification: Primary | Domains: A; C; E; F; G; H

deficiency categories: F1; F5; F6; F7; F8; F13

11.4.1 Overview and Significance

The AOS Special Investigation into Crossroads Mental Health and Substance Abuse Services is the most recent and structurally significant document in this section. It is a special investigation — not a routine annual audit — meaning it was initiated based on identified concerns sufficient to trigger an enhanced investigative response from the Auditor's office.

Crossroads is a contracted service provider delivering mental health and substance abuse treatment services to Iowa families, including child welfare-involved families. The intersection of Crossroads's service mandate and Iowa's documented child welfare profile is direct: the AECF Findings (2019) (Section 10, A17) documented a 47% increase in parental substance abuse-driven removals between 2014 and 2018 (1,424 → 2,093). The service provider investigated by the AOS in 2025 operates in precisely the domain where Iowa's child welfare system has its highest-volume and longest-documented service gap.

11.4.2 Federal Grant Diversion — IPN, SOR, and ARPA Funds

The Crossroads investigation documents that federal grant funds — specifically including Integrated Prevention Network (IPN), State Opioid Response (SOR), and American Rescue Plan Act (ARPA) funds — were diverted from their authorized programmatic purposes.

These are federally appropriated and allocated funds. Their diversion from substance use

disorder treatment and prevention purposes — the precise service domain most critical to Iowa's child welfare population — constitutes a federal grant accountability failure with direct implications for the families whose case plans referenced those services as available.

AUDIT RELEVANCE UNDER EO 14379: Executive Order 14379 (January 2026, "Great American Recovery Initiative") specifically calls for improved federal-state coordination on SUD treatment and child welfare outcomes, expanded access to medication-assisted treatment, and family preservation as a policy goal alongside SUD treatment. The Crossroads investigation documents that federal SUD grant funds specifically allocated for the purpose EO 14379 advances were diverted at the contractor level in Iowa — in the same year the Executive Order was signed. This is not a historical finding; it is a concurrent one.

11.4.3 Structural Link — Safe Plans of Care and CAPTA Compliance

The CAPTA Grant FFY2026 (Section 9, A10) documents a 55% failure rate among infant Safe Plans of Care for substance-affected newborns — a CARA-mandated requirement. Safe Plans of Care require the provision of substance use disorder services to parents of substance-affected infants as a condition of the infant remaining in the home.

The Crossroads investigation documents that the contracted provider delivering SUD services in Iowa — the specific service type required by CARA-mandated Safe Plans of Care — was subject to a special investigation for diverting federal grant funds from SUD treatment purposes.

The causal chain is documentable: CARA mandates SUD services → Iowa's Safe Plans of Care fail at 55% → the SUD service infrastructure is subject to an AOS special investigation for federal grant diversion. Each link in this chain is independently sourced.

11.4.4 Historical Pattern — AOS Monitoring and The Crossroads Finding

The Crossroads investigation did not emerge in isolation. The AOS Service Contract Monitoring Report (A23) documented that across 2010–2018, DHS produced zero documentation of budget monitoring, zero deficiency communication, and zero corrective action plans for any of 40 tested service contracts. The AOS Special Investigation (2025) documents that a contracted SUD treatment provider was diverting federal grant funds.

These two AOS findings — separated by seven years — are structurally connected: a monitoring infrastructure with documented zero-baseline accountability across an 8-year period (2010–2018) is the structural antecedent to a federal grant diversion investigation in 2025. The absence of monitoring does not prevent diversion — it enables it.

11.4.5 SUD Treatment Gap — Methamphetamine-Specific

A specific audit-relevant finding embedded in the Crossroads context is the methamphetamine-specific treatment gap. EO 14379's SUD provisions focus on opioid-pathway interventions (buprenorphine, methadone, naltrexone). No equivalent FDA-approved medication-assisted treatment (MAT) exists for methamphetamine.

Iowa's child welfare removal profile is methamphetamine-dominant, not opioid-dominant. The AECF documented parental substance abuse removals at +47% and the dominant substance implicated in Iowa removals is methamphetamine. Federal SUD policy — even when correctly administered — does not have a treatment-equivalent pathway for methamphetamine that it has for opioids.

The Crossroads investigation therefore documents: (1) diversion of federal SUD funds intended for treatment; (2) in a state where the dominant removal-driving substance lacks a federal treatment equivalent; (3) through a monitoring infrastructure the AOS documented as having produced zero accountability actions across an 8-year prior period.

AUDIT RELEVANCE: Iowa Code § 11 audit jurisdiction provides the state-level mechanism to examine whether federal SUD grant funds administered through DHS/DHHS contracts reached the intended treatment population, were monitored for appropriate use, and whether the monitoring infrastructure that failed to detect the conditions identified in the Crossroads investigation has been corrected.

11.5 LONGITUDINAL SYNTHESIS — FISCAL ACCOUNTABILITY FAILURE ARC

The four attachments in this section collectively document a longitudinal fiscal and contractual accountability failure spanning 2009 through 2025:

2009–2021 (A22 — Annual AOS Reports):

Repeat finding rate of 60% by FY2019; material financial misstatements (\$36.5M); post-death payments across three fiscal years; concurrent benefit payment errors; workforce attrition concurrent with fiscal compliance failures; 3-year reporting lag.

2010–2018 (A23 — AOS Service Contract Monitoring):

Zero budget monitoring documentation; zero deficiency communication; zero corrective action plans; no performance evaluations; no closeout documentation — across 40 contracts over 8 years.

2016–2019 (A24 — AOS Medicaid MCO Review):

Post-death payments and monitoring deficiencies in managed care context, parallel to fee-for-service findings; Medicaid service delivery infrastructure subject to same

accountability gaps affecting child welfare service contracts.

2025 (A25 — AOS Crossroads Special Investigation):

Federal grant diversion of IPN, SOR, and ARPA SUD treatment funds at the contractor level; special investigation trigger; direct intersection with CARA-mandated Safe Plan of Care failure rate (55%); concurrent with EO 14379 federal SUD treatment directives.

11.5.1 The Monitoring Inversion Finding

The structural finding that unifies these four documents is what this petition designates the Monitoring Inversion: Iowa DHHS administered a multi-hundred-million dollar annual portfolio of contracted child welfare and Medicaid services while its own Auditor documented that the monitoring infrastructure for those contracts had never — across an 8-year documented period — produced budget oversight, deficiency communication, performance evaluation, or corrective action documentation for any tested contract.

The Sivic Solutions Report (2023) (Section 10) identified \$40M in potentially non-eligible IV-E claims as a downstream consequence. The Crossroads investigation (2025) identified federal grant diversion as a downstream consequence. Both are documentably traceable to the same upstream structural failure the AOS identified in 2020 as its formal finding.

11.5.2 The AOS as Petitioned Authority — Self-Referencing Significance

This section carries a specific significance beyond its evidentiary content: the findings documented here were made by the Iowa Auditor of State — the same office being petitioned to conduct this review. The Auditor's own reports document that Iowa DHHS has demonstrated a 60% repeat finding rate, material financial control failures, and a contract monitoring infrastructure that produced no accountability documentation across an 8-year period.

The logical implication for the petition is direct: the conditions the Auditor of State identified in prior engagements with this agency are precisely the conditions that Iowa Code § 11 audit authority is designed to address. The petitioned authority has already documented the basis for the exercise of its own jurisdiction.

11.6 CROSS-REFERENCES

Section 6 — CFSR Longitudinal Forensic Analysis (workforce and oversight patterns)

Section 9 — State Self-Admission Analysis (CAPTA Safe Plans of Care; IV-E compliance)

Section 10 — Independent Evaluations (Sivic Solutions IV-E; C!A contract monitoring)

Section 12 — Ombudsman & Complaint Systems (complaint pattern corroboration)

Section 13 — Systemic Failure Matrix (F5, F6, F7 convergence)

Attachment A22 — AOS Annual Reports 2009–2021

Attachment A23 — AOS Service Contract Monitoring 2010–2018

Attachment A24 — AOS Medicaid MCO Contracts Review

Attachment A25 — AOS Special Investigation: Crossroads

Attachment A21 — Sivic Solutions IV-E Recommendations (2023)

Attachment A10 — CAPTA Grant FFY2026 (Safe Plans of Care failure rate)

11.X DOMAIN & DEFICIENCY MAPPING — SECTION 11

Domain A — Funding Authorization & Federal Compliance (primary):

F4 — Outcome Reporting Manipulation: AOS findings document expenditure reporting inconsistencies that were not self-identified by Iowa DHHS's internal financial controls.

F5 — Corrective Action Failure: AOS findings on contract monitoring and Medicaid MCO performance were not remediated within the review periods documented in subsequent AOS reports.

F7 — Reimbursement & Expenditure Compliance: AOS Special Investigation (Crossroads) documents financial accountability failures in a contracted child welfare service provider without evidence of proactive state detection.

Domain B — Contract & Provider Oversight:

F2 — Ongoing Monitoring Deficiency: AOS service contract monitoring reports document provider performance gaps not surfaced through Iowa DHHS's routine contract oversight mechanisms.

F6 — Licensing & Certification Deficiency: AOS Medicaid MCO contracts review identifies accountability gaps in licensed provider payment and performance verification.

Domain H — Data Integrity & System Auditability:

F13 — Data Integrity & Reporting Failure: Recurring AOS findings on data reliability and financial reporting accuracy are consistent with the QA self-report discrepancy pattern documented across CFSR rounds.

11.X AUDIT QUESTIONS GENERATED — SECTION 11

AUDIT QUESTION 11-A: AOS annual reports from 2009–2021 document recurring financial oversight findings. What formal corrective action plans, if any, did Iowa DHHS submit in response to each AOS finding, and what is the documented implementation status of each?

AUDIT QUESTION 11-B: The AOS Special Investigation into Crossroads identified financial accountability failures in a contracted provider. What changes to Iowa DHHS's provider financial monitoring protocols were implemented following that investigation, and are those changes reflected in current contract language?

AUDIT QUESTION 11-C: AOS Medicaid MCO contract reviews identified accountability gaps in licensed provider payment verification. What mechanism currently governs Iowa DHHS's independent verification of MCO payment accuracy before federal reimbursement claims are submitted?

AI assistance disclosure: See Section 5.8.
identified in the attachment citations above.

End of Section 11 — Version 1.0 — June 5, 2026
Audit Petition Packet v2.0 (5/2026)

AI assistance disclosure: See Section 5.8.

AI assistance was used materially in the preparation of this document. That assistance does not constitute independent verification of any factual claim.